FREE FAMILY EDUCATION · GUIDE 03

Breastfeeding with Confidence

Your First Weeks of Breastfeeding

A supportive learning workbook for preparing to feed, noticing patterns, and finding skilled help when needed.

Printable workbook · US Letter

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A supported start, without a perfect script

Feeding is learned over time by both baby and parent. Families may breastfeed, provide expressed milk, use formula, or combine approaches. Your goals and your baby's wellbeing both matter; asking for help is a practical step, not a failure.

> **Education, not individual medical advice**
>
> This guide cannot assess milk transfer, weight, hydration, or a feeding problem. Contact your baby's clinician and a qualified lactation professional for individual concerns. Seek urgent care for a seriously unwell baby.

A NOTE FROM TEESEE

A Note from Teesee: Gentle, informed support can make space for questions and for each family's feeding decisions.

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Prepare before baby arrives

If breastfeeding is part of your plan, discuss it during prenatal care. Ask how your birth location supports early feeding, whom to call after discharge, and how to access an International Board Certified Lactation Consultant (IBCLC) or other qualified lactation professional.

Questions for my care team

- How can I reach lactation support after birth?
- What if feeding is difficult overnight or on a weekend?
- When will baby have a weight and feeding check?
- What signs mean I should call the pediatric clinician?
- What options fit my health and feeding goals?

Colostrum is the first milk, produced in small amounts. Newborn stomach capacity and feeding needs change; a clinician can help interpret your baby's intake and growth. Avoid relying on a single sign in isolation.

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Early cues and feeding patterns

Possible early feeding cues

- Stirring or waking
- Bringing hands toward the mouth
- Turning the head or opening the mouth
- Seeking or rooting movements

Crying can be a later cue. When you notice early cues, offer a calm opportunity to feed if that fits your plan. In the early weeks, many newborns feed frequently; CDC describes 8–12 feeds in 24 hours as common for breastfed newborns. Individual guidance matters, especially after premature birth or medical concerns.

> **Follow your baby's care plan**
>
> Ask your baby's clinician how often to feed and when to wake for feeds. Do not use this workbook to set or delay feeding intervals.

Feeding observation log

Date/time	Cue noticed
Feed/support offered	Swallowing or other observation
Questions for clinician	

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Positioning and latch: what to notice

For a comfortable position, bring baby close and aligned, with head and body facing you. A lactation professional can demonstrate positioning and help assess latch. Some signs often discussed include a wide-open mouth, lips turned outward, chin close to the breast, and visible or audible swallowing.

Checklist

- [ ] Ask for hands-on guidance from a qualified professional
- [ ] Notice whether the parent is comfortable and able to support baby's body
- [ ] Ask what a deep latch and effective feeding may look and sound like
- [ ] Request another position if something feels uncomfortable

> **Pain deserves attention**
>
> Some tenderness can occur, but ongoing pain, damaged nipples, difficulty latching, or concern about intake merits timely support from a clinician or qualified lactation professional. This guide cannot determine the cause.

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Notice patterns; bring questions

A feeding log can help you explain what you are seeing. Clinicians combine feeding history with diaper output, physical assessment, and weight trends. A doula may help you record observations and find the right professional; a doula does not diagnose low supply, dehydration, jaundice, or oral restrictions.

Concern tracker

What I noticed and when	Baby's feeding/diaper pattern
Parent comfort or symptoms	Who I contacted and when
Advice and next follow-up	

Contact the baby's clinician promptly about

- Baby is difficult to wake for feeds or feeding suddenly changes
- You are concerned about wet/soiled diapers or weight
- Baby appears unwell, has breathing difficulty, or develops a fever
- You are worried baby is not getting enough milk

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Common questions and skilled support

Build a support team

- Baby's pediatric clinician
- Parent's obstetric or primary care clinician
- IBCLC or other qualified lactation professional
- Trusted feeding support person

Ask for specific help

- Can someone observe a full feed?
- Can you explain what you are assessing?
- When should we follow up?
- Who do we call if the plan is not working?

Lactation and care contacts

Name/role	Phone or portal
Hours/after-hours	When to contact

A doula can offer nonclinical education, encouragement, and practical household support. Refer clinical questions to the appropriate licensed or qualified provider.

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Make feeding support practical

Feeding support plan

What the feeding parent wants nearby

Who brings water/food

Who manages household tasks

Who tracks questions or appointments

How family will support the chosen feeding plan

Checklist

- [ ] Keep care-team numbers easy to find
- [ ] Plan how to ask for help during nights and weekends
- [ ] Share preferences about visitors and feeding privacy
- [ ] Make space for rest and hydration without prescribing a feeding outcome

Reflection

What kind of support would feel respectful and useful to me?

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Share the work around feeding

A feeding parent can benefit when another caregiver owns nearby tasks. Agree what support is welcome and how to ask before changing or supplementing a feeding plan. Questions about supplementation, pumping, or medicines belong with a qualified clinician or lactation professional.

Feeding support plan

Who brings water or a meal

Who handles diapering or settling after a feed

Who tracks clinician questions

How the household will protect feeding privacy

Who can help if feeding feels difficult

Checklist

- [ ] Use encouraging, nonjudgmental language
- [ ] Follow the feeding parent's stated goals
- [ ] Help schedule or attend professional appointments if invited
- [ ] Keep a list of trusted after-hours contacts

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Your feeding plan can change

Families make feeding decisions with their values, health circumstances, and professional advice. Changing a plan can be a thoughtful response to new information. You deserve respectful support whichever safe feeding approach you choose.

Optional reflection · 2 Corinthians 12:9

A reference for readers who welcome Christian encouragement; it is not a promise of a particular feeding outcome.

Sources and further reading

Newborn breastfeeding basics
Centers for Disease Control and Prevention

Getting a good latch
U.S. Office on Women's Health

Considering breastfeeding
Centers for Disease Control and Prevention

Educational information reviewed against these sources on October 14, 2026. Confirm current guidance with your qualified care team.

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